

This is a general guideline and does not represent a professional care standard governing providers' obligations to patients. Care is revised to meet individual patient needs. This is a quality improvement document and should not be part of the patient's medical record.

UNC PICU Code Sepsis Pathway

Triggers to consider sepsis:

- Altered mental status
- Changes in perfusion (mottled, cool skin, delayed cap refill, etc)
- Weak or bounding pulses
- Acute changes in vital signs

Meets criteria for **positive sepsis screen**?

Patient has at least 2 of the following:

1. Fever or hypothermia
2. High risk condition such as lines and tubes, malignancy, s/p transplant, technology dependence, immunosuppression, etc
3. Abnormal HR or RR
4. WBC > 15,000 or < 5,000

If yes, then bedside RN to call PICU fellow for **sepsis huddle**.

If patient deemed septic, proceed with **sepsis bundle**:

- Use EPIC pediatric sepsis order set to order labs (CBCd, CRP, critical care ABG, Bcx), fluids and antibiotics (see recommended Abx from link to sepsis pathway in the order set)
- Unless pharmacy is immediately available at the bedside, call 4-4111 to ask for code sepsis to PICU or text CODE SEPSIS PEDIATRIC PAGER and provide patient name, MRN, and weight
- Provide supplemental O2
- Obtain access if necessary
- Reassess and provide additional fluid boluses as necessary
- Document in EPIC using .sepsispedeval smartphrase

Fellow to **notify** Afsaneh by email (afsaneh_pirzadeh@med.unc.edu) for positive screens for tracking purposes

Goals are for cultures to be obtained, resuscitation to have occurred, and antibiotics to be administered within one hour of sepsis huddle.